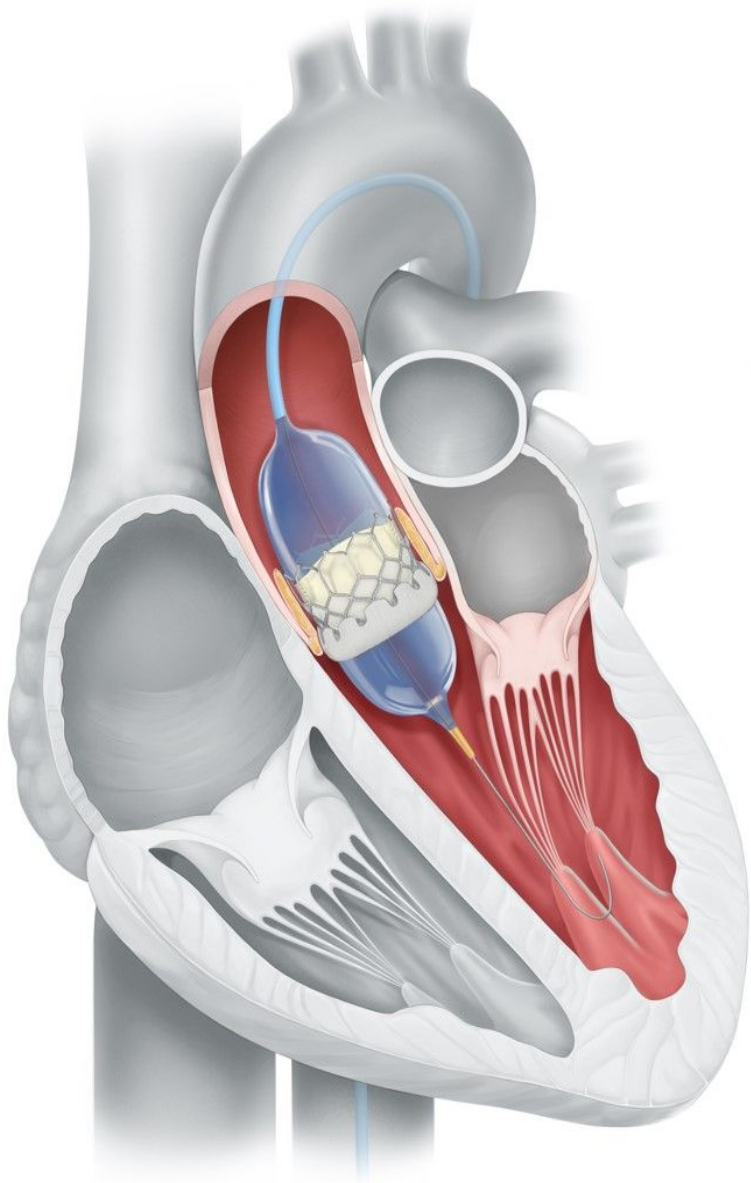




Post-TAVI Nursing Care & Monitoring

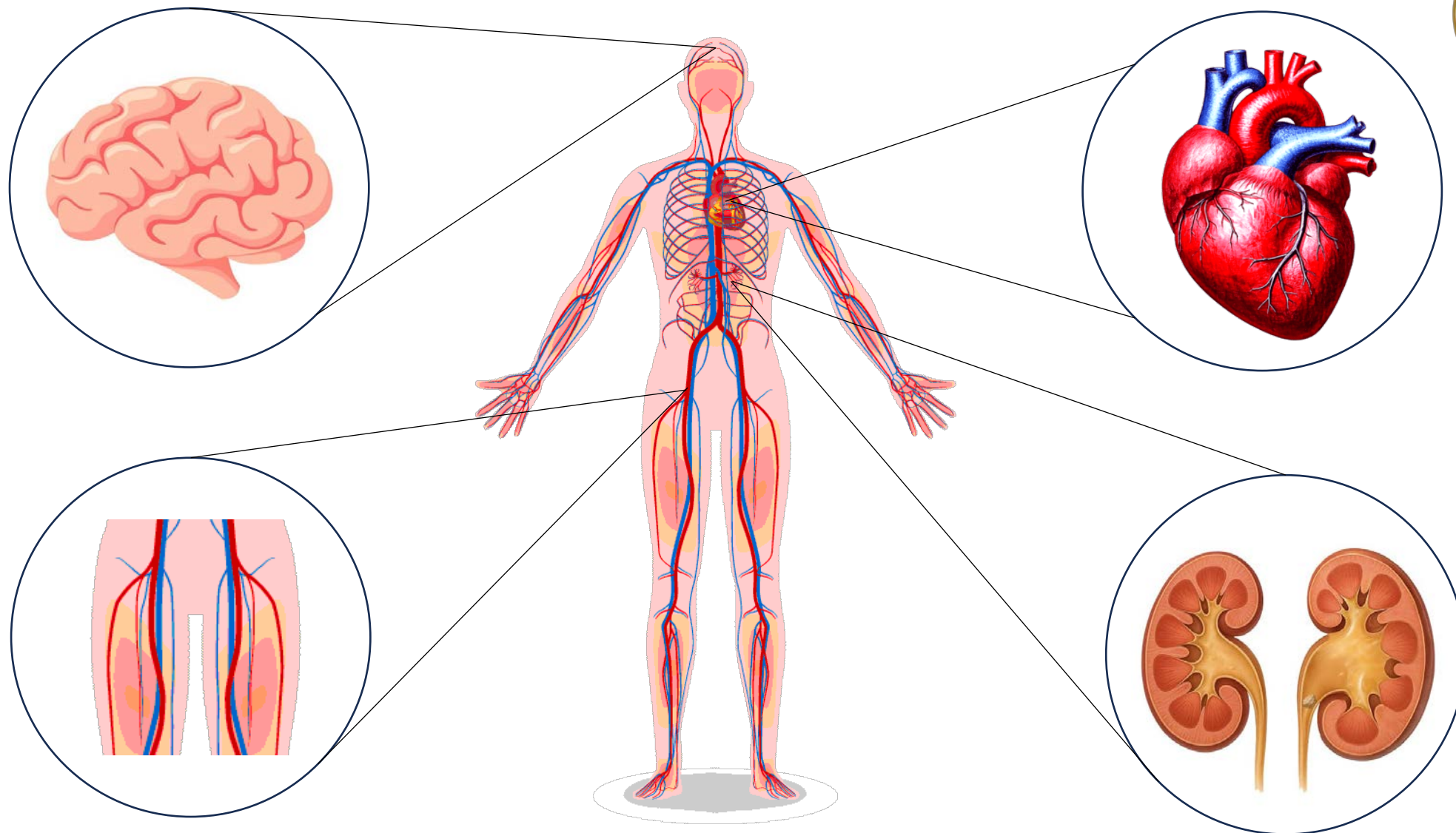
**Pattanaporn Supina
CCU. Siriraj Hospital**

Outline



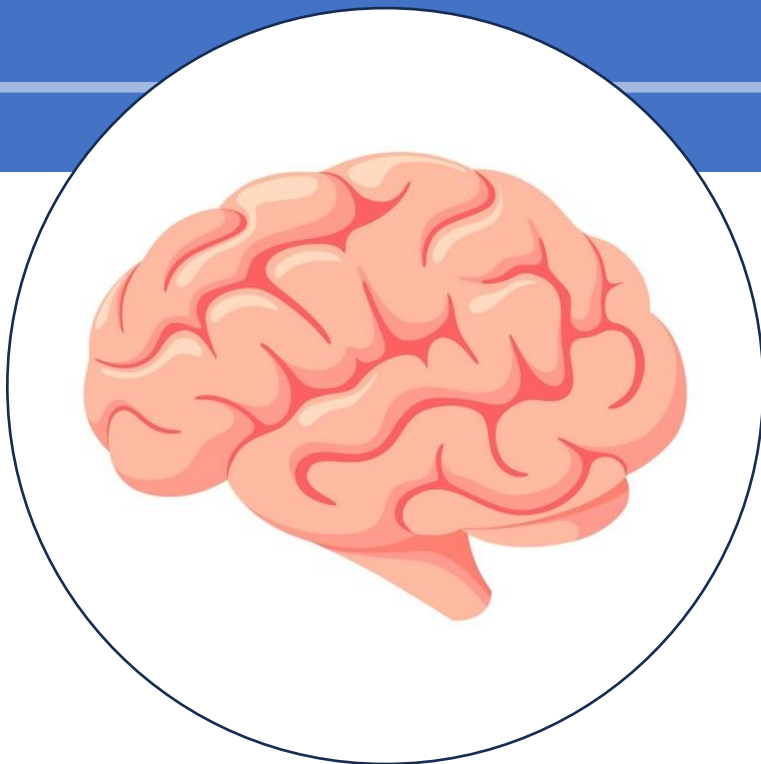
- Immediate Post – TAVI Phase
- Recovery & Pre – discharge Phase





Post-TAVI Nursing Care & Monitoring

Stroke



- **Patients & Procedural Risk Factors^{1,2}**
 - TIA, DM, DLP, PAD, severe valve calcification, low LVEF, AF
 - Procedural risks: hypotension, prolonged procedure time, ACT



Stroke



- **Assessment**

- A focal or global neurological deficit with at least one of the following: altered consciousness, motor/sensory deficits, speech disturbance, visual loss, or other neurological signs.³ → **NIHSS / GCS**
- Record last seen normal time immediately



Stroke

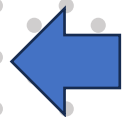
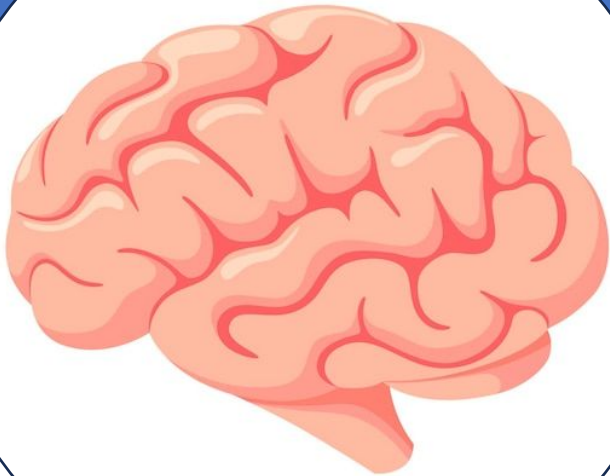


- **Monitoring** ⁴

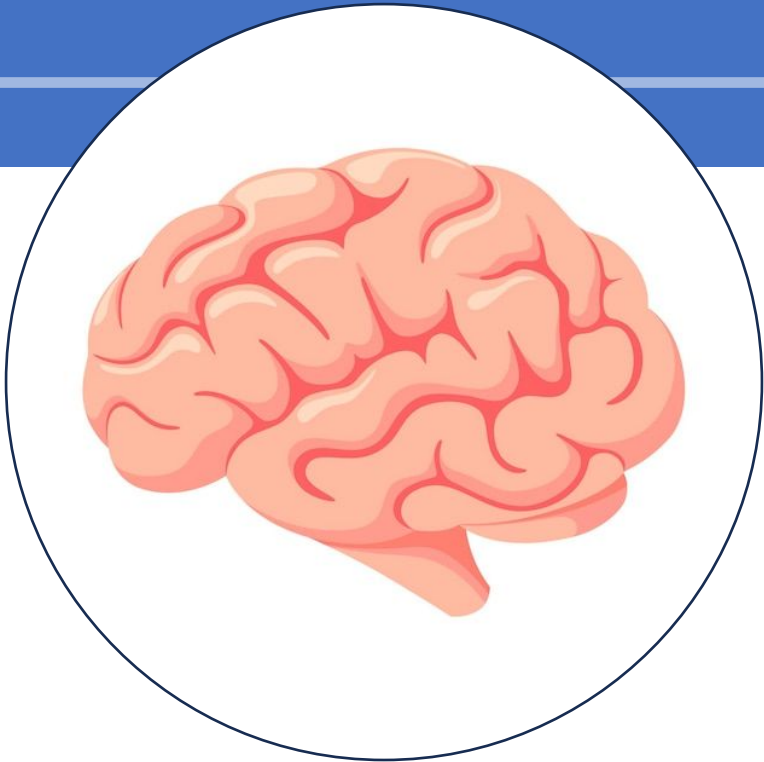
- ABC: Airway, Breathing, Circulation
- Normothermia
- Blood glucose level in a range of 140 -180 mg/dL
- Repeated neurological assessment

- **Action**

- Notify the heart team & Activate Stroke fast tract



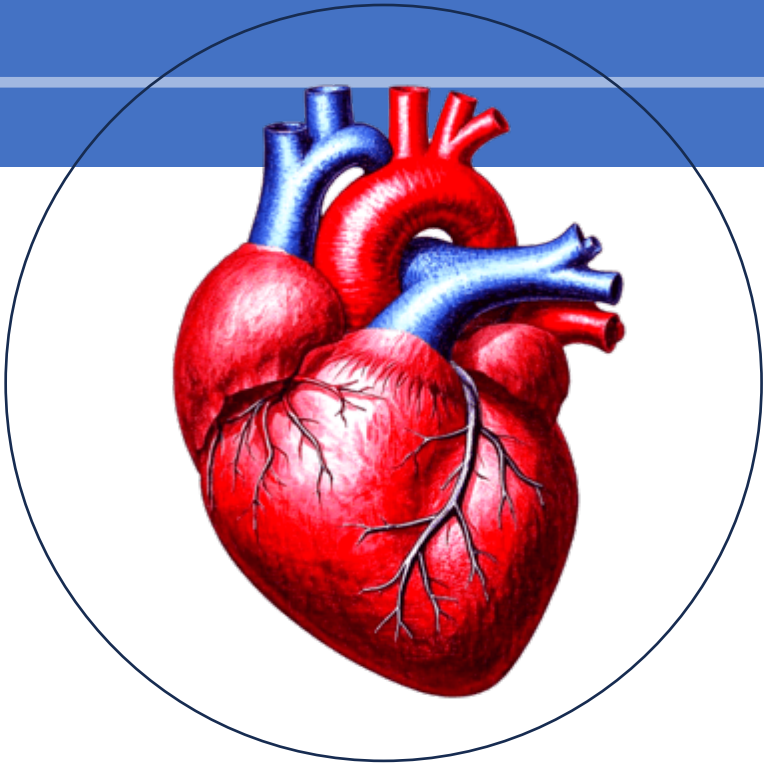
Delirium



- **ICU Delirium Assessments**
;Confusion Assessment Method-ICU (CAM-ICU)
- **Correct the Underlying Cause**
- **Non-pharmacological Management Strategies**
- **Pharmacological Management Strategies**



Heart Conduction

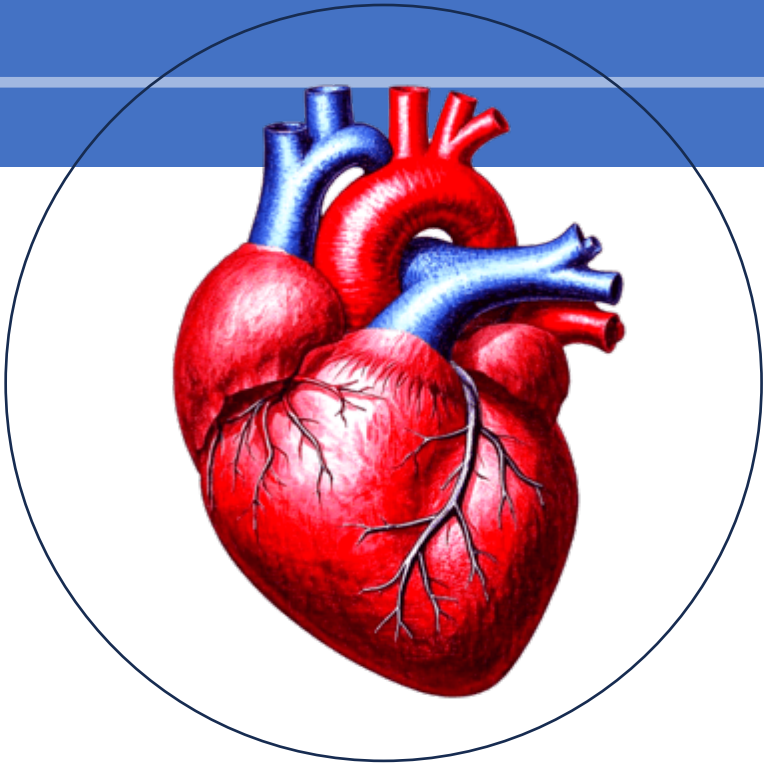


- **Patient Information & Procedural Characteristics ⁵**

- Baseline ECG
- Aortic Valve Calcification
- Valve Type
- Implantation Depth
- Valve Oversizing



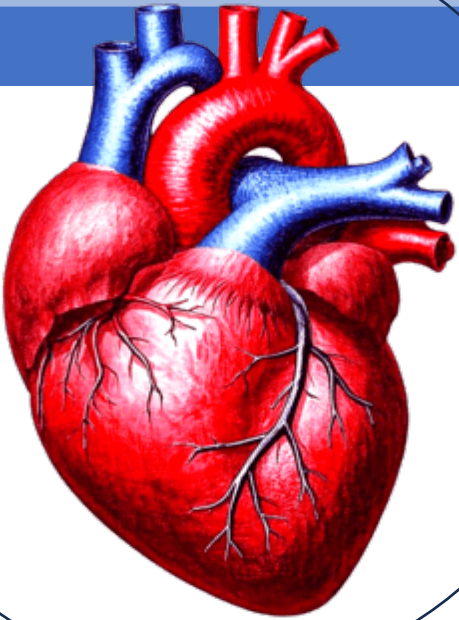
Heart Conduction



- V/S & EKG monitoring and 12 – lead ECG; compare with previous ECG
- Preparation for Potential AV Block; verify temporary pacemaker function & prepare the emergency cart.
- **EP Consultation if PPM implantation is required**

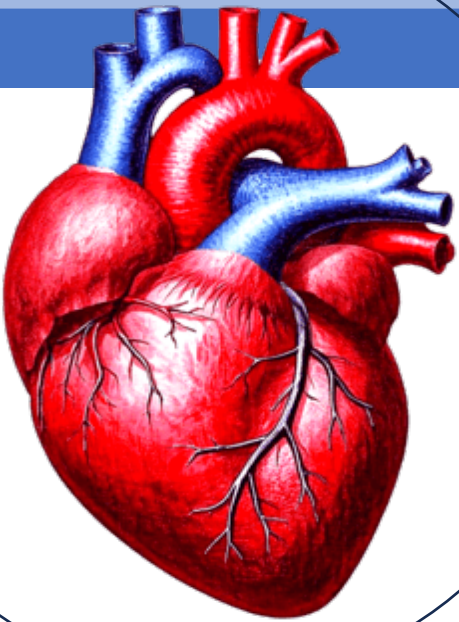


PVL & Annulus rupture

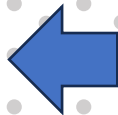


- Patient Information & Procedural Characteristics
- Assess signs of heart failure or cardiac tamponade
- Continuous ECG & Hemodynamic monitoring

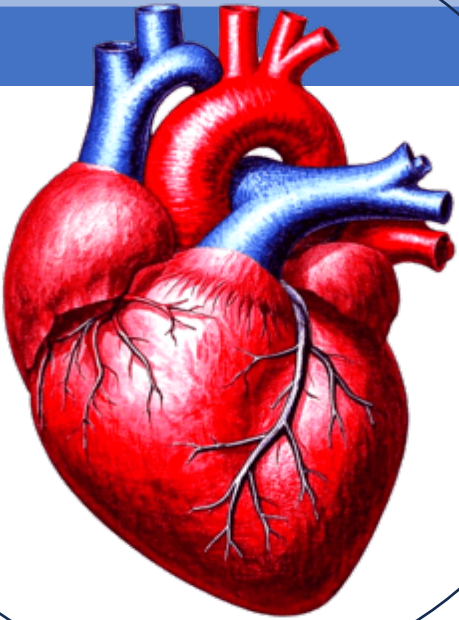
PVL & Anulus rupture



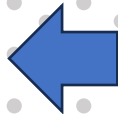
- Maintain Hemodynamic Stability
- Prepare Emergency Cart & Resuscitation equipment
- Notify the heart team immediately



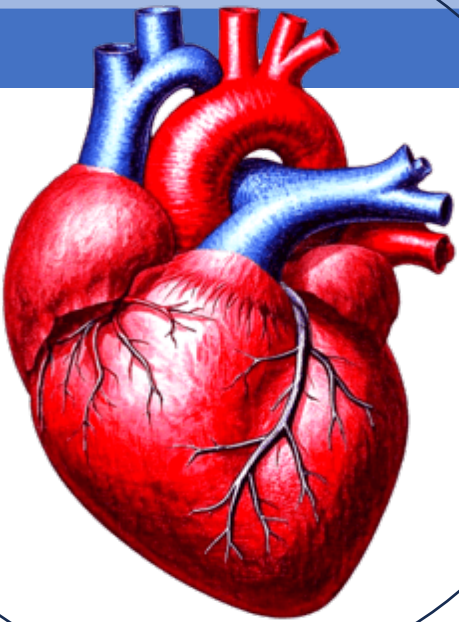
Coronary obstruction



- Patient Information & Procedural Characteristics
- Assess Ischemic Signs, Symptoms, and Hemodynamic Changes
- Echocardiography: Monitor for New Severe LV Dysfunction



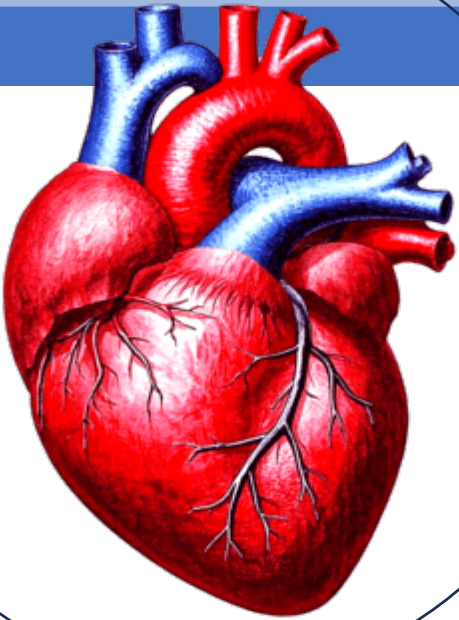
Coronary obstruction



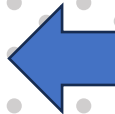
- Obtain 12-Lead ECG and Compare with Baseline ECG
- Continuous ECG and Hemodynamic Monitoring
- Maintain Hemodynamic Stability



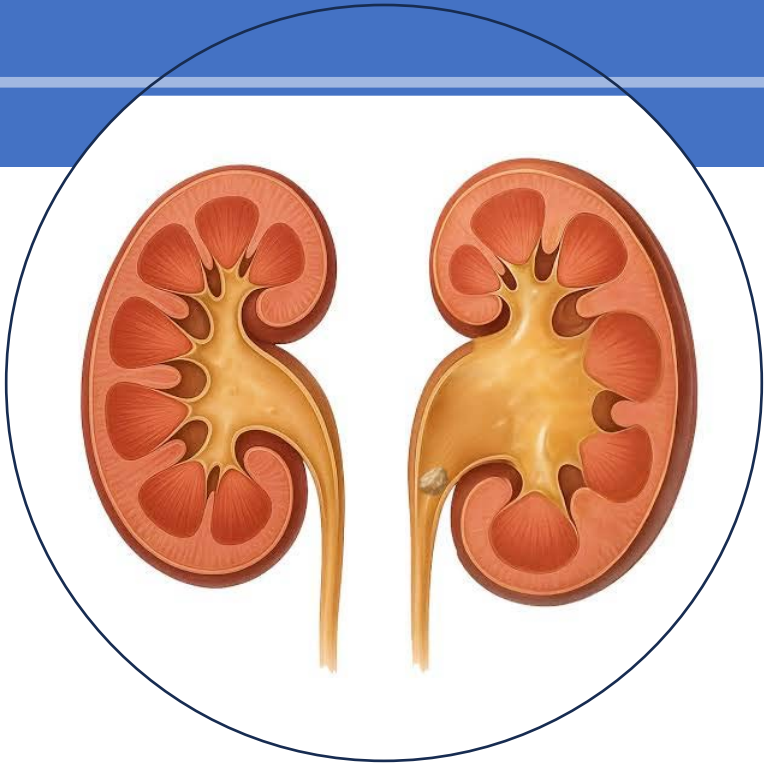
Coronary obstruction



- Notify the Heart Team Immediately
- Prepare for Emergency Coronary Intervention



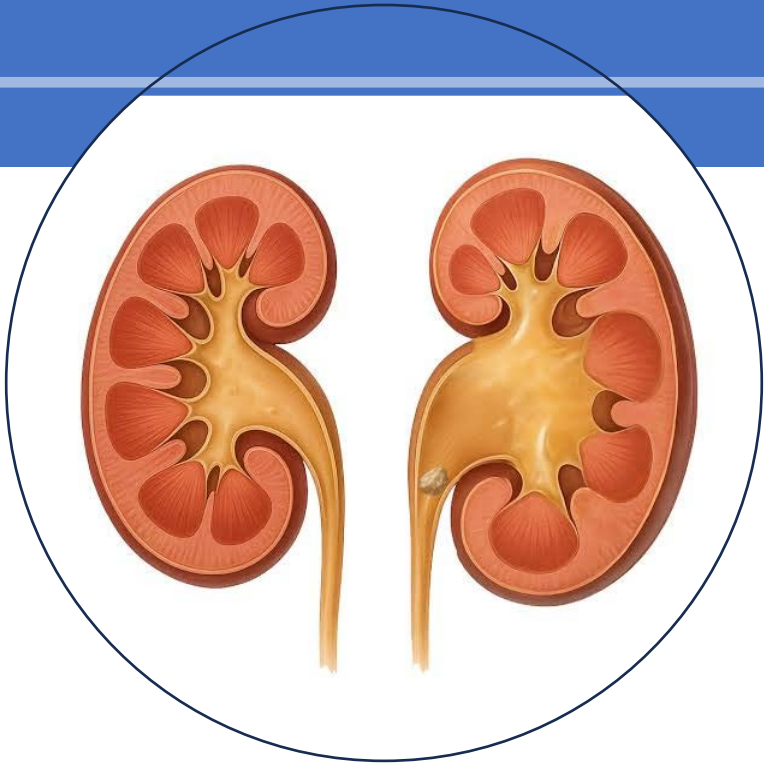
CIN^{6,7}



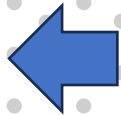
- Patient Information & Procedural Characteristics
- Maintain adequate hydration
- Maintain blood pressure and prevent hypotension



CIN



- Monitor intake & output (I/O)
- Monitor serum creatinine
- Consult Nephrologist

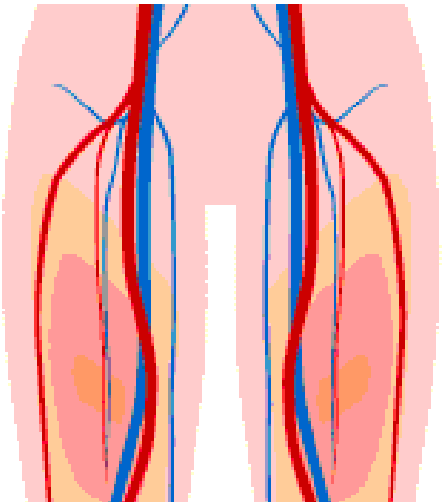


Vascular

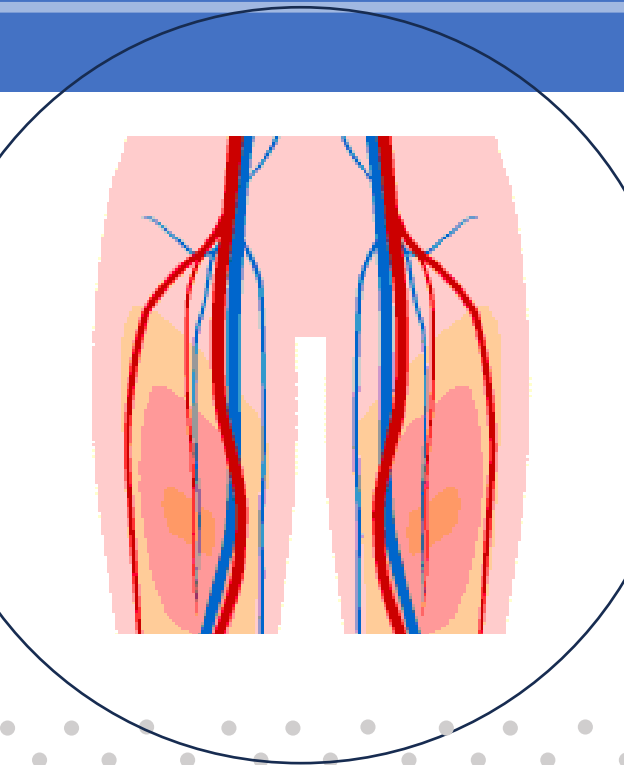


Assess Access Site & Check Distal Pulses & Limb Perfusion ^{7,8}

Vascular Complications	Signs & Symptoms
Hematoma	- Swelling & Palpable skin hardening around puncture site
Retroperitoneal Hemorrhage	- Moderate to severe back pain, Ipsilateral flank pain, Vague abdominal/back pain
Pseudoaneurysm	- Large ecchymotic, painful, pulsatile swelling at the insertion site - Bruit and thrill heard in the groin
Arteriovenous Fistula	- Swollen, tender extremity - Continuous bruit and/or thrill at access site
Arterial Occlusion/Emboli	- Pain, Paresthesias, Pulselessness, Pallor or Poikilothermia / coolness



Vascular



- Assess Hemodynamic Stability
- Maintain Hemostasis
- Blood transfusion & coagulation correction
- Maintain Bed Rest and Follow Post-Procedure Instructions
- Notify Heart Team for Abnormal Findings

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